

TRIDENT HEALTH DIAGNOSTICS

3600 Clinical St, Norfolk, VA 23510 | CLIA #: 37D8899003 | Tel: (757)555-0310

LABORATORY REPORT

Patient: Kazimiera Kowalczyk	DOB: March 16, 1959 (Age 66)
MRN: MRN-3500324	Gender: Female
Ordering Physician: Dr. Jovan Petrovic, MD — Infectious Disease	Accession #: VG-10605488
Specimen Type: Venous Blood / Urine	Collected: October 31, 2025 at 12:00 AM
Received: October 31, 2025 at 01:00 AM	Reported: October 31, 2025 at 08:00 AM

COMPLETE BLOOD COUNT (CBC) WITH DIFFERENTIAL

Test	Result	Units	Reference Range	Flag
WBC — White Blood Cell Count	30.4	x10 ³ /μL	4.5 – 11.0	CRIT-H
RBC — Red Blood Cell Count	3.27	x10 ⁶ /μL	3.80 – 5.10	L
Hemoglobin	12.1	g/dL	12.0 – 16.0	
Hematocrit	31.6	%	36.0 – 46.0	L
MCV	83.2	fL	80.0 – 100.0	
MCH	19.7	pg	27.0 – 33.0	L
Platelets	131	x10 ³ /μL	150 – 400	L
Neutrophils	80.1	%	50.0 – 70.0	H
Lymphocytes	35.2	%	20.0 – 40.0	

COMPREHENSIVE METABOLIC PANEL (CMP)

Test	Result	Units	Reference Range	Flag
Glucose (Fasting)	193	mg/dL	70 – 99	H
BUN	77	mg/dL	7 – 25	H
Creatinine	3.18	mg/dL	0.50 – 1.10	H
eGFR	77	mL/min	> 60	
Sodium	139	mEq/L	136 – 145	
Potassium	3.9	mEq/L	3.5 – 5.0	
Calcium	11.5	mg/dL	8.5 – 10.2	H
AST	130	U/L	10 – 40	H
ALT	666	U/L	7 – 56	H
Total Bilirubin	5.4	mg/dL	0.1 – 1.2	H
Albumin	5.1	g/dL	3.5 – 5.0	H

THYROID FUNCTION PANEL

Test	Result	Units	Reference Range	Flag
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TSH	7.86	mIU/L	0.45 – 4.50	H
Free T4	1.47	ng/dL	0.82 – 1.77	
Free T3	4.6	pg/mL	2.0 – 4.4	H

CLINICAL INTERPRETATION / COMMENTS

Severe leukocytosis with left shift consistent with bacteremia or systemic infection. Elevated creatinine indicates early acute kidney injury in the context of suspected sepsis. Blood cultures x2 are recommended. Empirical broad-spectrum antibiotics and IV fluid resuscitation should be considered.

■ **CRITICAL VALUES COMMUNICATED — See header for details.**